

THE ALBERTA DISABILITY SYSTEM BREAKDOWN

April 2026

What Disabled Albertans Are Being Asked to Decide Without the Information They Need to Decide It

*Information Asymmetry as the Modern Form of Paternalism
in Alberta's Disability System*

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I.

Introduction

This report examines a structural feature of the Alberta disability system that is rarely named directly and almost never named as a feature. The feature is information asymmetry: the systematic gap between what the province knows and discloses, and what the people whose lives are governed by that knowledge are permitted to know.

The transition from the Assured Income for the Severely Handicapped program (AISH) to the Alberta Disability Assistance Program (ADAP) on July 1, 2026 is the most visible current expression of this gap. Approximately 50,000 current AISH recipients are scheduled for transfer to ADAP, drawn from a total AISH caseload of 79,290 after exempted categories are subtracted.¹

The eligibility criteria, the assessment methodology, the appeal mechanisms, the continuation periods for supplementary benefits, and the per-recipient transition timelines are, as of the date of this report, either undisclosed or communicated only through inconsistent secondhand accounts at frontline offices. Recipients are being asked to accept a fundamental restructuring of their income security without access to the information that would let them evaluate it.

This is not, however, a new condition. It is the most recent stage of a pattern that operates across the entire lifecycle of a disabled person in Alberta. The argument advanced in this report is that information asymmetry — the deliberate or institutionalized withholding of information from the people whose lives that information governs — is the operating principle of disability administration in this province, and that it is the direct functional successor

¹Government of Alberta, Assisted Living and Social Services — AISH Caseload Open Data, September 2025; cited and disaggregated in The Alberta Disability System Breakdown, We're Still In This Together (April 2026). The figure of 79,290 represents the total AISH caseload as of September 2025. Of this population, the cohort being transitioned to ADAP on July 1, 2026 is approximately 50,000, after exempted categories (PDD recipients, recipients over 60, continuing care residents, and palliative or terminal medical conditions) are subtracted from the total. The exact ADAP-bound population is unconfirmed because the government has not published palliative or non-PDD severe developmental disability counts.

to the paternalism that defined the Sexual Sterilization Act era from 1928 to 1972.

The legal architecture of eugenics in Alberta has been dismantled. The repeal of the Sexual Sterilization Act in 1972, the Leilani Muir judgment in 1996,²

and the subsequent compensation processes formally ended the regime that authorized the involuntary sterilization of approximately 2,832 Albertans deemed unfit to reproduce.³

The information-control mechanism that sustained that regime, however, was never specifically dismantled. It was the practice of treating the affected person as not entitled to the information that would let them participate in decisions about their own life. The decisions of the Alberta Eugenics Board were not communicated to those it surveilled. The medical files of those sterilized were sealed from them, in many cases for decades. The legal authority under which the Act operated was unintelligible to most of those subject to it. The mechanism was not the surgery. The mechanism was the asymmetry.

This report identifies that mechanism in five distinct stages of contemporary disabled life in Alberta. It does not argue that the Government of Alberta is sterilizing disabled people. It argues that the structural relationship between the institutional holders of information and the people whose lives are reshaped by that information is, in 2026, materially the same as it was in 1962. The form has changed. The function has not.

The five stages examined are: (1) prenatal and reproductive education; (2) early childhood and the recognition of developmental difference; (3) adult

²Erika Dyck, *Facing Eugenics: Reproduction, Sterilization, and the Politics of Choice* (University of Toronto Press, 2013). The figure of approximately 2,832 sterilizations under the Sexual Sterilization Act 1928–1972 is drawn from documented Eugenics Board records as compiled by the Eugenics Archives Canada project (eugenicsarchive.ca, University of Alberta). Source figures across the scholarly literature range between approximately 2,800 and 2,832 depending on counting methodology and records access at the time of compilation.

³*Muir v. The Queen in Right of Alberta* (1996), 132 D.L.R. (4th) 695 (Alta. Q.B.). Plaintiff Leilani Muir was sterilized at the Provincial Training School at Red Deer in 1959 at age 14 on the authority of the Alberta Eugenics Board. Her successful 1996 civil action against the Government of Alberta opened the records of the Eugenics Board and led to subsequent compensation processes for additional plaintiffs.

diagnosis and the navigation of fragmented support systems, including the structural asymmetry of documentation that the AISH-to-ADAP transition exposes; (4) environmental and occupational health information; and (5) succession — the structural information vacuum surrounding the future of a disabled adult after the death or incapacitation of a primary caregiver. Each stage is examined with reference to publicly available information, to the documented operational practices of the relevant ministries, and to the lived experiences of disabled Albertans and the families who support them.

The report concludes by articulating what disabled Albertans are owed — not as a matter of policy preference, but as a precondition for the consent that any restructuring of disability income support, or of any disability program, must rest on in order to claim political legitimacy.

II.

The Argument in Brief

The province's disability administration is structured around a presumption that the affected person is best served by receiving information at the moment of administrative necessity, and not before. Information about reproductive risk is offered when an at-risk pregnancy is identified, not as part of comprehensive education prior to reproductive decision-making. Information about developmental disability is offered when a clinician makes a diagnosis, not as part of public health literacy that would equip parents to recognize what they are seeing. Information about adult support programs is offered upon application, not as part of the public information available to a person attempting to determine whether to apply. Information about the operational rules of ADAP is, to the extent it has been offered at all, offered to a recipient at the moment they are processed through the transition — not in advance of it, when refusal or appeal might still be meaningful.

The cumulative effect is a population whose decisions about their own bodies, their own children, their own livelihoods, and their own deaths are made on the basis of information the province possesses but does not share. That is not a neutral administrative choice. It is the operating mechanism of a system that does not regard the affected person as the primary user of the information about their own life.

The repeal of the Sexual Sterilization Act did not end this orientation. The transition from AISH to ADAP confirms that it remains intact. This report is an attempt to name it.

III.

Stage One — Reproductive and Prenatal Information

The information environment surrounding reproduction in Alberta presents disability through a narrow and selective window. Routine prenatal screening is offered for a small set of conditions — most prominently Trisomy 21 (Down syndrome), Trisomy 18, Trisomy 13, and structural anomalies detectable by second-trimester ultrasound. This screening is funded, normalized, and integrated into the standard prenatal care pathway. Genetic counseling, where it is offered, is offered primarily to expectant parents over 35, to families with already-identified hereditary conditions, or to those whose initial screens return abnormal results.

Information that is not made systematically available within this framework includes: the neurodevelopmental conditions that constitute the largest category of childhood disability diagnoses in Canada (autism spectrum disorder, ADHD, learning disabilities, intellectual disabilities not associated with the conditions screened for); the chromosomal contributions of paternal as well as maternal lineage, despite documented evidence that paternal genetic factors contribute substantively to autism heritability; the lived realities of raising a disabled child within Alberta's specific support architecture, including Family Support for Children with Disabilities (FSCD) wait times and Persons with Developmental Disabilities (PDD) program eligibility thresholds; and the basic vocabulary and recognition criteria that would equip a prospective parent to identify developmental difference when it appears.

The Alberta Health Services prenatal information materials, the Calgary and Edmonton zone parenting education programs, and the publicly available Alberta Education curricula on reproductive health do not contain this information at any stage of structured delivery. A prospective parent in Alberta who relies on the public information system — the system funded and administered by the province — to prepare them for the realities of disability in their child's life will reach the moment of becoming a parent without the information necessary to recognize what they are looking at.

This is not a failure of individual clinicians, educators, or public health staff, who in many cases provide such information when asked. It is a failure of structured availability. The information is not absent from human knowledge. It is absent from the channels through which the public encounters the public information system.

The consequence of this gap is borne by the families who walk into it. A parent whose child begins to miss developmental milestones is told by a pediatrician — accurately, within the constraints of what that pediatrician is able to say without a diagnostic process — that “some children develop later.” This is true. It is also incomplete in a way that has predictable downstream effects. By the time a developmental difference is formally identified, often years later, the family has lost the early-intervention window that public health research consistently identifies as critical, and has typically also lost the opportunity to apply for FSCD funding before the program’s documented multi-year wait time begins to accrue. The information that would have allowed earlier action existed at every point. It was not delivered in a form the family could use.

The structural relationship is identifiable. The province possesses information about the prevalence, presentation, and developmental trajectories of childhood disability. The province possesses information about the support programs available, their eligibility thresholds, and their wait times. The province does not communicate this information in the prenatal and early-parenting period when it would meaningfully shape the decisions a family makes — when a family might prepare emotionally and financially for the possibility, when a family might begin documenting milestones in a way that would later support a faster diagnostic referral, when a family might apply early to programs whose wait times exceed the period during which intervention is most effective. The information is delivered, instead, at the point of administrative contact. The system, in effect, waits for the family to encounter the consequences of the information vacuum before filling the vacuum, and then fills it only to the extent necessary to complete the immediate administrative task.

The historical resonance is not incidental. In the Sexual Sterilization Act era, the eugenics framework operated on the principle that ordinary citizens were

not equipped to evaluate disability information accurately and that decisions about disability and reproduction were properly made by experts. The contemporary version is softer in form — the province does not claim authority over reproductive decisions — but operates on a related principle: that comprehensive disability information is not necessary for ordinary citizens, and that the public information system need not be organized around equipping them to make such decisions deliberately. The historical version produced involuntary sterilization. The contemporary version produces what might be called uninformed reproduction — not in the sense that any reproductive choice is wrong, but in the sense that the choice is being made without the information the province is in a position to provide.

A society that takes informed consent seriously begins providing the information for that consent before the moment of decision arrives. Alberta does not. The information vacuum at the prenatal stage is the first stage of the lifecycle pattern this report examines. It is not the most consequential stage. It is, however, the stage that establishes the principle: that the affected person is treated by the public information system as someone to whom information will be provided when administrative necessity requires it, and not before.

IV.

Stage Two — Early Childhood and the Recognition of Developmental Difference

The information asymmetry that begins before reproduction continues into early childhood, where it is compounded by the structure of pediatric primary care in Alberta. Family physicians and pediatricians are the first institutional point of contact for a parent observing developmental difference in a young child. They are also, in most cases, the only point of contact during the period in which intervention is most effective. The information environment they operate within shapes what families learn, when they learn it, and what they are equipped to do with that learning.

The structural condition of pediatric primary care in this province is that family physicians and pediatricians are generally not equipped to deliver developmental disability information at the level of specificity that families require. This is not a critique of individual practitioners. It is a description of the system in which they work. The Royal College of Physicians and Surgeons of Canada and the College of Family Physicians of Canada do not require developmental disability assessment expertise as a core competency for general pediatric or family practice. Specialist developmental pediatricians exist in Alberta in numbers radically disproportionate to demand: developmental pediatric assessment in Alberta is constrained by a number of practitioners radically disproportionate to demand, with documented multi-month and in some cases multi-year waits across health zones. The general pediatrician or family physician is, in practice, the gatekeeper to specialist assessment, and the criteria they apply to determine whether a referral is warranted are not standardized across the province.

The result is a recognition gap. A parent who reports that a child is missing developmental milestones is told, accurately within the constraints of generalist practice, that “some children develop later” and that “we will continue to monitor.” This is medically defensible advice in the absence of clear red flags. It is also, for the substantial subset of children whose developmental differences are real and consequential, advice that delays diagnosis by months or years. By the time a developmental concern becomes

severe enough to warrant specialist referral, the child has typically aged out of the early-intervention window and has accrued substantial wait time before specialist assessment can occur.

This recognition gap operates inside a broader information environment that does not provide families with the tools to advocate around it. The province does not publish, in any public-facing document accessible to parents in the prenatal or early-parenting period, a list of developmental milestones with the specificity that would allow a parent to recognize concerning patterns at the age at which they first appear. The province does not publish the criteria its publicly funded specialist services use to determine referral appropriateness. The province does not publish wait time data for developmental pediatric assessment by health zone. A family attempting to advocate for earlier referral or assessment is doing so without any of the information that would let them present their case in terms the system is structured to receive.

The downstream programs through which a child's eventual diagnosis must be processed in order to access support are similarly opaque to families until the moment of contact. The Family Support for Children with Disabilities (FSCD) program is the primary provincial mechanism for funding intervention services, respite, and aide support for families of disabled children. Its eligibility criteria, application process, and current wait times are not publicly displayed in any form a prospective applicant can review before applying. Families learn that FSCD exists, in most cases, only when a clinician mentions it during or shortly after diagnosis. Families learn the wait time, in most cases, only after they have submitted an application. The wait time itself is documented at an average of three years province-wide as of the January 2025 Inclusion Alberta survey of 540 families across 79 of 87 Alberta electoral districts.⁴

⁴Inclusion Alberta, "Experiences with Family Support for Children with Disabilities" (FSCD Survey Report, January 2025); 540 respondents from 79 of 87 Alberta electoral districts. The 3-year average wait for full FSCD services is province-wide and reflects the cumulative wait across the program's three-stage access process implemented in 2023. See also Alberta Auditor General, Report on FSCD (2022), documenting caseworker training and oversight failures; Hold My Hand Alberta (September 2024), documenting one caseworker carrying 400+ families against a recommended maximum of 100.

A child diagnosed at age four whose family applies on the day of diagnosis will, on average, not access FSCD-funded supports until that child is seven. The early-intervention research that justifies the existence of FSCD identifies the ages of two to five as the window of greatest developmental plasticity. The program is structured such that it cannot, in its current operational form, deliver supports during the period its own evidentiary basis identifies as critical.

The Persons with Developmental Disabilities (PDD) program, which provides adult support to those with intellectual and developmental disabilities, presents a parallel information opacity at the transition from youth to adult services. Eligibility for PDD requires a documented developmental disability with onset before age 18, evidence of significant adaptive functioning impairment, and, where applicable, an IQ assessment consistent with the diagnostic standard for intellectual disability. The threshold criteria and the assessment process are not publicly documented in a form accessible to families during the years in which they are preparing for the transition. Families discover, often within months of their child's eighteenth birthday, that the supports their child has received under FSCD will not continue under PDD, that the application process is independent and lengthy, and that there is no continuity-of-service mechanism between the two programs.⁵

Children whose disabilities do not meet PDD's intellectual-developmental disability threshold but who require ongoing support — children with autism without intellectual disability, children with ADHD with significant adaptive impairment, children with mental health conditions, children with chronic medical conditions producing functional impairment — are not eligible for PDD at all and have no provincial adult support program available to them. This is not communicated to families in advance. Families discover the gap when they reach it.

⁵Inclusion Alberta and ALIGN, "PDD Current State: Critical and Urgent: Experiences with Persons with Developmental Disabilities" (January 2025); Government of Alberta, Seniors, Community and Social Services 2024-25 Annual Report, documenting 13,641 PDD recipients. The Government of Alberta has not published PDD waitlist data since 2021. Alberta.ca PDD eligibility documentation requires a documented developmental disability with onset before age 18, evidence of significant adaptive functioning impairment, and — where applicable — IQ assessment consistent with the diagnostic standard for intellectual disability under the DSM-5 / ICD-11.

The cumulative effect across the early childhood and youth-to-adult transition periods is a population of disabled Albertans and their families operating without the information necessary to plan, advocate, or prepare. The province possesses comprehensive information about all of these systems. The province does not communicate that information until the moment of administrative contact. Families repeatedly discover, at each stage of their child's development, that information that would have allowed earlier action existed, was held by the institutional bodies whose programs they were navigating, and was not made available to them when it would have changed the outcome.

This is the early-childhood expression of the lifecycle pattern. It is not a different mechanism than the one identified in Stage One. It is the same mechanism — selective communication of information at the moment of administrative necessity, and not before — operating on a population that has now entered the system. The pattern continues into adulthood, where it acquires further dimensions specific to the disabled adult's own navigation of supports the same system was designed not to make legible to them in advance.

V.

Stage Three — Adult Diagnosis, Fragmented Adult Supports, and the Asymmetry of Documentation

The information asymmetry that defines the experience of disabled children and their families does not resolve when the disabled person reaches adulthood. It changes form. The disabled adult who reaches the threshold of adult services in Alberta encounters a fragmented landscape of programs, each with its own eligibility criteria, application processes, assessment mechanisms, appeal rights, and information requirements. None of these are integrated. None are documented in a form that allows a person to evaluate the full system before entering any part of it. The disabled adult navigates this system, in most cases, alone or with the assistance of family members who are themselves operating without comprehensive information.

A substantial population of disabled adults in Alberta receives diagnosis in adulthood for conditions present throughout childhood. Adult-onset diagnosis is particularly common for autism spectrum disorder in women and gender-diverse individuals, for ADHD across all demographics, for learning disabilities masked in childhood by compensatory coping strategies, and for neurodevelopmental conditions whose presentations were not recognized within the diagnostic framework operative at the time of the individual's youth. The Canadian medical literature documents this phenomenon extensively. The implications for support access are significant: an adult who lived an undiagnosed disabled childhood entered adulthood without FSCD support, without PDD support if they would have qualified, and without the documented diagnostic history that the contemporary AISH application process requires.

The AISH program — the Assured Income for the Severely Handicapped — is the primary provincial income support for adults whose disabilities substantially limit their ability to earn a livelihood. Its eligibility criteria, codified in the AISH Act and the AISH Policy Manual, require a permanent medical condition, substantial limitation of ability to earn a livelihood, and the absence of likely improvement through training, treatment, or rehabilitation. The application requires extensive medical documentation, typically

including a physician's medical report, supporting specialist documentation, and adjudication by a provincial medical assessor. The application process is documented at multiple months under standard conditions, with substantial regional and case-specific variation. Appeals of denials are heard by the AISH Appeal Panel under the program's existing structure, with a further pathway to the Citizens Appeal Panel — a pathway being eliminated under Bill 12 for AISH eligibility decisions effective with the ADAP transition.⁶

What is not documented publicly, in a form a prospective applicant can review before applying, includes: the specific criteria the medical adjudicator applies to determine "substantial limitation of ability to earn a livelihood"; the evidentiary thresholds for "permanence" of a condition; the assessment standards for fluctuating conditions, which do not present consistently across an assessment period; the criteria for determining "likely improvement," a determination that requires the adjudicator to make a prognosis the applicant's own physicians may not have made; and the operational practice of the program with respect to mental health conditions, neurodevelopmental conditions, chronic pain conditions, and conditions whose presentation does not match the framework the adjudicator was trained within.

The applicant is therefore preparing an application without knowing the criteria against which it will be evaluated. The applicant is documenting a condition without knowing what level of documentation will be considered sufficient. The applicant is describing functional impairment without knowing how the assessor weighs the categories of impairment described. The applicant is, in many cases, doing this while experiencing the very symptoms that constitute the disability for which they are applying — cognitive impairment, fatigue, executive function impairment, mental health symptoms, chronic pain — and is doing so without the support of a trained advocate, because no such free advocacy service is integrated into the application process at the provincial level.

⁶Bill 12, the Financial Statutes Amendment Act, Alberta Legislature, passed December 9, 2025; section 12.8 removes appeal of the AISH-to-ADAP transition and renders Medical Review Panel eligibility determinations final, with no further appeal to the Citizens Appeal Panel. See also Inclusion Alberta Media Release, November 26, 2025; Zachary Weeks, "Bill 12 and ADAP: What Alberta's New Disability Law Means for AISH Recipients," November 28, 2025; CBC News (Bellefontaine), August 21, 2025, reporting on the ADAP Discussion Guide.

The result is a documented denial pattern. The Citizens Appeal Panel overturned approximately one in three AISH eligibility decisions on appeal during the period for which records are available, with the overturn rate exceeding 40% in 8 of 19 documented quarters between fiscal years 2021-22 and 2025-26. The denial-and-appeal cycle is not a quality assurance mechanism. It is a documented filter that removes from the program a population of eligible applicants whose disabilities specifically impair their capacity to navigate the appeal process. The most disabled applicants are the most likely to be filtered out by a process whose information requirements they are least equipped to meet.⁷

For the adult who is approved for AISH, the information environment does not improve. The AISH Policy Manual is a public document, but it runs to several hundred pages, written in legal-administrative language, and is not paired with any plain-language version that a recipient might use to understand their own rights and obligations under the program. The cohabitation rules — the provisions that determine how a partner's income affects benefit calculation — are documented in the manual but are routinely misunderstood by recipients and, in documented cases, by frontline AISH staff. The earnings exemption rules are similarly opaque: a recipient who works part-time may be eligible to retain a portion of earned income up to a documented threshold, but the calculation is complex, the threshold is subject to change without comprehensive recipient notification, and the deduction practices vary in their consistency. Recipients regularly report receiving deductions they did not anticipate, on bases they cannot reconstruct from the documentation provided.

The supplementary health benefits program associated with AISH — covering prescription medications, dental care, optical care, and certain medical equipment — operates under its own coverage formulary that is partially documented and partially not. Recipients learn what is covered when they attempt to access it. The transition from AISH-tier supplementary benefits to the supplementary benefits structure under ADAP, effective July 1, 2026, has

⁷AISH Appeal Overturn Report (April 2026), *The Alberta Disability System Breakdown*. Aggregated quarterly data shows the Citizens Appeal Panel overturned approximately one in three AISH eligibility decisions on appeal, with the overturn rate exceeding 40% in 8 of 19 documented quarters between fiscal years 2021-22 and 2025-26.

not been comprehensively documented. Recipients have not been provided with a list of medications, devices, or services that will be covered under the new structure. Recipients are being asked to plan for a transition whose effects on their healthcare access they cannot evaluate because the information has not been made available.

The adult disabled population in Alberta is therefore navigating a system in which: the eligibility criteria for the program providing their income support are not fully documented; the program manual governing their daily compliance with that program is too long and too technical for most recipients to use; the cohabitation, earnings, and benefit-calculation rules are routinely misapplied even by program staff; the supplementary health coverage they depend on is not comprehensively documented; the transition to the successor program is being implemented without disclosure of the rules that will govern it; and the appeal pathway that has historically allowed recipients to challenge decisions made about them is being eliminated for the transitioning cohort.

This is not a system designed to serve a population of disabled adults navigating it with the support of comprehensive information. It is a system designed around the presumption that the recipient will accept the determinations made about them by adjudicators applying criteria the recipient cannot review, will comply with rules the recipient cannot fully understand, and will lose the benefits to which they are entitled when their navigation of the system fails — not because they are ineligible, but because the information they would need to remain eligible was not made available to them.

The transition from AISH to ADAP exposes a further dimension of this asymmetry that requires direct examination. The transition is structured around an exemption framework: certain categories of AISH recipients are designated as automatically remaining on AISH and not subject to reassessment. As of the publication of this report, the government has identified four such exempted categories: recipients of the Persons with Developmental Disabilities (PDD) program (13,641 individuals as of the 2024–25 SCSS Annual Report); recipients over the age of 60 (13,603 individuals as of September 2025 ASLSS open data); residents of continuing

care homes (2,423 individuals as of September 2025 ASLSS open data); and recipients with palliative or terminal medical conditions outside continuing care (count unpublished). The remaining recipients — a population approximately 50,000 strong, the precise figure of which depends on the unpublished palliative count and on whatever further exemption adjustments the government may make before July 1, 2026 — are designated for default transfer to ADAP, with reassessment by the Medical Review Panel as the only pathway back to AISH.⁸

The structure of this exemption framework reveals something the framework's design was not intended to disclose. For the exempted populations, the government is treating the recipient's existing AISH file — the documentation produced by the same provincial assessment system, adjudicated by the same medical assessors, signed by the same physicians, governed by the same eligibility criteria — as conclusive evidence that the recipient is permanently and severely disabled. No reassessment is required. No new medical report is required. No appearance before a Medical Review Panel is required. The existing file is sufficient.

For the transitioning population, the same kind of file, produced by the same system, is treated as insufficient. The recipient is moved by default to ADAP. Their reinstatement to AISH requires submission of a new Disability Assistance Medical Report and adjudication by a Medical Review Panel hired by the same ministry that benefits financially from each finding of ADAP placement. The independent appeal pathway that historically existed for AISH eligibility decisions has been eliminated for this cohort. Two recipients with substantively identical files — one in an exempted category, one not — receive structurally different treatment. The variable is not the file. The variable is the category the government has assigned to the recipient.

⁸Government of Alberta, ADAP Program Page (alberta.ca/alberta-disability-assistance-program); cross-referenced with Government of Alberta SCSS 2024-25 Annual Report (PDD recipients = 13,641); ASLSS Open Data, September 2025 (recipients over 60 = 13,603; continuing care residents = 2,423). The palliative/non-PDD severe developmental disability category count is not published. Total exempted population is therefore at least 29,667; the ADAP-bound transitioning population is therefore at most approximately 50,000, with the precise figure dependent on unpublished palliative counts and any further exemption adjustments.

This contradiction makes the operative criterion legible. The Medical Review Panel is not assessing whether a transitioning recipient's disability is real or severe. The existing file already established that, and the government has accepted the equivalent established record for every recipient in an exempted category. The Medical Review Panel is, in functional effect, assessing whether a recipient can be plausibly framed as a candidate for employment. The exemption categories themselves identify this: recipients over 60 are exempted because their employment trajectory is no longer considered relevant; PDD recipients are exempted because their developmental disability is considered conclusive of unemployability; continuing care residents are exempted because their living situation is conclusive of unemployability; palliative recipients are exempted because their prognosis is conclusive. Each exemption category is defined not by the severity of the recipient's disability — every AISH recipient by definition has a severe permanent disability — but by the implausibility of framing the recipient as employable. The transitioning cohort is the cohort the government considers framing-amenable.

The assessment criterion the Medical Review Panel will apply has not been published. The standards by which "framing-amenable" will be operationalized into a determination have not been disclosed. The recipient whose file is being newly evaluated under this unpublished criterion has no opportunity to review what is being applied to them, no opportunity to respond to it, and no independent body to which an erroneous application can be appealed. The criterion is being applied by appointees of the ministry whose budget benefits from each ADAP placement, in the absence of the appeal panel that historically reviewed decisions of this kind. The government's existing record on the recipient — the record establishing that the same government previously found this person unable to sustain a livelihood — is held in the same ministry's filing cabinet as the new determination is being made.

This is not a disability program transition. It is a labour-market reclassification of disabled persons, conducted by a panel hired by the ministry with the financial interest in the outcome, with the appeal mechanism removed before the reclassification begins, against an existing

record that contradicts the new determination, under criteria the affected population cannot review.

The labour market into which this reclassification is occurring requires examination. The Alberta general unemployment rate as of February 2026 is 6.3%, with full-time employment having decreased by 17,300 positions in a single month. The number of unemployed Albertans in 2024 was 189,400, a 24% year-over-year increase. The youth unemployment rate in Alberta in 2024 was 14.5%, an increase of 2.3 percentage points. The unemployment rate for Albertans with disabilities in 2024 was 8.9%, in a province where the employment rate gap between persons with and without disabilities is 16.8 percentage points. The employment rate for persons with severe disabilities — the cohort most comparable to AISH recipients — is 26.4% nationally. These are not theoretical figures. They describe the labour market the Medical Review Panel's framing-amenable population is being directed into.⁹

The same Government of Alberta that is preparing to reclassify approximately 50,000 disabled recipients as candidates for employment introduced, on April 1, 2026, the Immigration Oversight Act. The Minister of Jobs, Joseph Schow, stated that the legislation is about ensuring that "Albertans have first crack at Alberta jobs." The legislation responds to a labour market condition the same government had been instrumental in producing: the number of work permit holders in Alberta increased from approximately 45,000 in 2021 to approximately 180,000 in 2025, a 300% increase representing 135,000 additional workers. As of January 1, 2026, the province had 271,024 non-permanent residents, approximately 60% of whom held work permits. In the business, finance, and administration sector — the category of clerical and office work that would be most accessible to disabled workers capable of seated employment — Temporary Foreign Worker positive Labour Market Impact Assessments increased from 166 in 2018 to 2,242 in 2023, an increase

⁹Alberta Job Market Snapshot, February 2026 (jobbank.gc.ca): Alberta general unemployment rate 6.3%; full-time employment decreased by 17,300 positions in February 2026; cumulative unemployed Albertans 189,400 in 2024 (a 24% year-over-year increase); youth unemployment rate Alberta 14.5% (a 2.3 percentage-point increase). Statistics Canada, "Labour Market Characteristics of Persons with and without Disabilities, 2024" (released May 14, 2025): national employment rate for persons with severe disabilities 26.4%; wage gap of \$2.22/hour (widening from \$1.91 in 2023). Job Bank Canada, Alberta Economic Scan 2024: Alberta PWD unemployment rate 8.9% (vs. 7.9% non-disabled); Alberta employment rate gap PWD vs. non-PWD 16.8 percentage points.

of 1,251%. The government's stated reason for the increase, on the record from Minister Schow, was that employers "cannot find the employees in Alberta."¹⁰

The two positions cannot both be true. A government that is legislating to ensure Albertans have first access to Alberta jobs is a government that has identified Albertan workers as being structurally bypassed by employers in their own labour market. The same government cannot simultaneously assert that 50,000 disabled Albertans, with documented permanent severe disabilities, will encounter a labour market capable of absorbing them. The labour market that has been recruiting internationally at twelve times the prior rate to fill clerical and administrative roles is not a labour market that has been preserving capacity for disabled workers whose access requires accommodation. The structural barriers that produced the TFW expansion — employer hiring practices, accommodation costs, productivity expectations, training infrastructure — are the same barriers that produce the documented disability employment gap. ADAP does not address those barriers. ADAP reclassifies the disabled population as ostensibly employable and dispatches that population into a market the government has already identified as failing to absorb its non-disabled job-seeking residents.

The operational principle is identifiable. The reclassification is administrative; the employment is hypothetical; the income reduction is real. The recipient whose existing AISH file is treated as insufficient is moved to ADAP. The recipient who cannot find sustainable employment in the labour market just described continues to receive the lower ADAP base benefit. The federal Canada Disability Benefit, designed to lift disabled persons above the poverty line, is clawed back by the province in a one-to-one offset. The

¹⁰CBC News (Lauren Johnson), "Alberta to compel employers hiring temporary foreign workers to register provincially," April 1, 2026: work permit holders in Alberta increased from approximately 45,000 in 2021 to approximately 180,000 in 2025 (a 300% increase); Minister of Jobs Joseph Schow stated the legislation is about ensuring "Albertans have first crack at Alberta jobs." Alberta Quarterly Population Report Q3 2024 (Treasury Board and Finance, December 2024): 271,024 non-permanent residents in Alberta as of January 1, 2026, approximately 60% holding work permits. CBC News (Karina Zapata), "Alberta employers rely more on temporary foreign workers for business roles," June 24, 2024: Temporary Foreign Worker positive Labour Market Impact Assessments in business, finance, and administration positions increased from 166 in 2018 to 2,242 in 2023, a 1,251% increase. Government's stated reason for the increase, on the record from Minister Schow: employers "cannot find the employees in Alberta."

supplementary health benefits structure under the new program is undisclosed. The transition benefit that maintains current AISH-rate income through December 31, 2027 expires without legislative protection thereafter. The recipient who was reclassified as framing-amenable on July 1, 2026 enters January 1, 2028 at the lower benefit rate, in the same labour market, with the same employment barriers, with no independent appeal, against the same existing file the same government had previously accepted as conclusive of permanent severe disability.

The exemption framework, far from constituting a protective structure, exposes the protective character of the existing AISH framework as conditional on administrative grace. A government that has the authority to designate which recipients are exempt has equally the authority to undesignate them. The exemption categories are not legislated. They are operational decisions of the ministry, communicated through public-facing program documentation that the ministry itself controls. The ministry has demonstrated, in the months preceding the July 1, 2026 transition, a documented willingness to alter operational parameters of the program without notice or announcement. The earned income exemption was reduced from \$1,072 to \$300 and subsequently adjusted to \$700 over the course of the transition planning period; recipients learned of these changes when they encountered them, not when they were made. The same operational discretion that allows the ministry to alter exemption thresholds without notice allows the ministry to alter exemption populations without notice. A PDD recipient counted as automatically exempt today may not be counted as automatically exempt on July 1, depending on how the operational definition of the PDD category is administered between now and then. The 13,641, 13,603, and 2,423 figures cited above are accurate as of the publication of this report. They are not protected. They are subject to reduction by administrative practice, without notice to the affected population.¹¹

¹¹Inclusion Alberta, ADAP Fact Sheet, October and December 2025; Government of Alberta ADAP Program Page (current). The earned income exemption for single AISH clients was historically \$1,072/month. Documentation in the transition period reflected a reduction to \$300/month, subsequently adjusted to \$700/month. Recipients learned of these changes through encounter with revised program documentation rather than through formal notice or announcement.

The relationship between the program and the recipient is structurally identical to the relationship the report identified at the prenatal stage and the early childhood stage: information is held by the institutional party, communicated at the moment of administrative necessity if at all, and the consequences of the information vacuum are borne by the affected individual. The form has changed — surgical sterilization is not at stake; the operative mechanism is income, healthcare access, and survival — but the principle is the same. The recipient is not regarded by the system as the primary user of the information governing their own life. The transitioning recipient is being moved from a program they qualified for, on the basis of a documented disability the same ministry adjudicated, by a panel applying an unpublished criterion, into a program whose operational rules have not been published, with no independent appeal, on the basis of an exemption framework whose categories the ministry can adjust without notice, into a labour market the same government has legislated as inaccessible to its existing workforce.

The transition from AISH to ADAP is the most consequential current expression of this dynamic, but it is not novel. It is the latest stage of a lifecycle pattern in which disabled Albertans, at every transition point of their lives, encounter the consequences of an information environment the province administers but does not share.

VI.

Stage Four — Environmental and Occupational Health Information

The information asymmetry pattern operating across the disability lifecycle in Alberta is not unique to disabilities present from birth or developmental disabilities recognized in childhood. It extends to acquired disability — to the chronic conditions, autoimmune disorders, organ system failures, and degenerative processes that result from environmental exposures, occupational exposures, and conditions whose origins in exposure are documented in the medical literature but are not communicated to affected individuals in a form that allows them to recognize the connection.

A substantial population of disabled Albertans became disabled through exposure to substances and conditions whose harmful effects were known to industrial, governmental, and medical institutions before, during, or shortly after the period of exposure, but were not communicated to the exposed population in time to prevent the harm. This is a different mechanism than the prenatal, early-childhood, and adult-navigation stages this report has examined. It is the same principle, operating at a different stage of life and through a different institutional channel.

Consider the case of a woman born in Alberta in 1964. Across her life, she developed Crohn's disease, rheumatoid arthritis, fatty liver disease, kidney failure, recurring kidney stones, ischemic heart disease, diabetic complications, polycystic ovary syndrome, and chronic muscle and joint pain. Each condition was assessed independently. Each generated its own diagnostic process, its own specialist referral chain, its own treatment plan. The conditions were not connected to each other in her medical care. They were treated as a series of unrelated chronic illnesses presenting in the same individual.

It took approximately twenty-five years across multiple specialists for the connecting framework to surface in her care: that the cluster of conditions she had developed was consistent with the documented health effects of long-term exposure to per- and polyfluoroalkyl substances (PFAS), asbestos, and

lead-based paint — exposures that were present in the workplaces, residences, and communities of her early adult life. The information that would have allowed earlier recognition existed, in primary scientific literature dating to the 1960s for asbestos, the 1970s for lead, and the 1980s and 1990s for PFAS. The communication of that information from research literature to clinical practice to public health education to individual patient awareness was systemically delayed across each transition. By the time she was able to access the framework that explained her conditions, she had been failed by the public health information system for the duration of an adult lifetime.¹²

This is one case, presented anonymously and without identifying detail, illustrative of a population. The same pattern operates across the cohort of Canadian veterans whose service-related exposures produced cascading chronic illness that took decades to be acknowledged within the Veterans Affairs Canada framework. The same pattern operates across the cohort of industrial workers in Alberta's oil and gas sector, whose long-term exposure to volatile organic compounds, hydrogen sulfide, and silica produced documented chronic health effects that were not consistently communicated to workers as risks at the time of exposure. The same pattern operates across the population exposed to leaded gasoline emissions during the period of leaded fuel use in Canada (1928-1990), whose neurological and cardiovascular effects are documented in the epidemiological literature but were not communicated to the exposed population in any form during the period of exposure. The same pattern operates across the populations affected by contaminated water on First Nations reserves in Alberta and across Canada, whose long-term health effects continue to be documented inadequately and communicated insufficiently to affected community members.

The institutional architecture of this information failure is distinct from the architectures examined in earlier sections of this report. Public health

¹²The Compound Failure, Pillar I (April 2026), *The Alberta Disability System Breakdown*. Aggregates documented Canadian regulatory and clinical evidence on PFAS, asbestos, lead, and related environmental and occupational exposure trajectories. See also *Indigenous Disability Alberta Complete Collection* (April 2026), Section 1, on Indian Hospitals and the Sixties Scoop as disability-creating institutional systems documented in the testimony record.

information about environmental and occupational exposure is held across federal departments (Health Canada, Environment and Climate Change Canada), provincial ministries (Alberta Health, Alberta Environment and Protected Areas, Workers' Compensation Board), municipal authorities, professional licensing colleges, and the research institutions whose primary publications generate the evidence base. The transmission of information from primary research to clinical practice to public awareness depends on a chain of professional and institutional actors, each of whom may or may not regard the transmission of the information as their responsibility. The result, in practice, is that exposed individuals encounter the information about their exposure — when they encounter it at all — only after the harm has manifested, often only after extensive personal effort to research their own conditions, and frequently only because of contact with another affected person or family who has done the same research independently.

The principle is structurally identical to the principles identified at the earlier lifecycle stages. The institutional system holds information that, if communicated to the affected population in a form they could use, would have changed the outcome. The institutional system communicates the information at the moment of administrative or clinical necessity, if at all, and not before. The cost of the information vacuum is borne by the exposed individual, who absorbs the harm and, if they survive long enough to receive a coherent diagnosis, often must construct the connecting framework themselves through personal research, advocacy networks, and the labor of family members who become unpaid investigators of their own loved ones' medical histories.

The connection between this stage and the disability income support stage examined earlier is direct. Many disabled adults applying for AISH and other adult support programs in Alberta have acquired their disabilities through exposures the province possessed information about but did not communicate. The applicant's medical documentation demonstrates the disability. The applicant's medical history demonstrates the exposure. The institutional connection between the exposure and the disability — the framework that would identify the disability as the predictable consequence of an information failure for which the institution bears partial responsibility

— is not part of the AISH adjudication process. The applicant is required to document their disability in clinical terms. The applicant is not invited to document the institutional failures that produced it.

This omission is not technical. It is structural. A disability income support system that adjudicated applications with reference to the institutional histories that produced the disabilities would be a system in which the province acknowledged its partial responsibility for the population it serves. The system as currently structured does not. The applicant approaches the program as a supplicant rather than as a citizen whose disability the institutional architecture of the province participated in producing. The relationship is asymmetric in the same direction as every other relationship this report has examined: the institutional party holds the information, the affected individual bears the cost, and the disclosure of the information to the affected individual occurs at the moment of administrative necessity rather than at the moment when disclosure would have prevented the harm.

The environmental and occupational health stage is, in this sense, the stage at which the lifecycle pattern reveals its full implications. Disability is not, in the experience of many disabled Albertans, a condition that simply exists in the individual. It is a condition produced, in part or in whole, by the operating practices of the institutional environment. The information that would have allowed the individual to recognize, prevent, or mitigate that production was held by the institutions that produced the condition. The disability income supports the institution offers the individual after the production are calibrated to keep the individual in poverty. The transition currently being implemented under Bill 12 is calibrated to remove from the income support population those individuals whose continued retention the institution does not consider fiscally justifiable.

The architecture of the harm is consistent. The information was withheld. The disability resulted. The support is conditional. The conditions are changing without consultation. The affected individual is asked to consent to changes the terms of which have not been disclosed. The relationship of the institution to the affected individual remains, at every stage, the relationship of an entity that regards itself as entitled to make decisions about a population that the entity does not regard as entitled to participate in those decisions.

This is the lifecycle pattern at its full extent. The remaining stage — the stage following the death or incapacitation of a primary caregiver — extends the pattern beyond the disabled individual's own lifetime, and is examined in the section that follows.

VII.

Stage Five — Succession, and What Happens After the Primary Caregiver

The information asymmetry pattern that operates across a disabled person's life does not end at that person's death. It extends past it, and it extends past the death of the primary caregiver who supported them. The final stage of the lifecycle pattern this report examines is the structural information vacuum surrounding the future of a disabled adult after the person who has been holding their life together is no longer there.

This stage is, in the experience of the parents and partners of disabled adults across Alberta, the stage that haunts the previous four. It is the stage that wakes parents at three in the morning. It is the stage that is rarely named in policy documents and almost never named in the public-facing materials of the institutions that will be responsible for the disabled adult after the caregiver is gone. The institutional silence on this question is not accidental. The institutions that will be responsible for those disabled adults have a documented operational and fiscal interest in not communicating clearly what that responsibility will look like in practice, because clear communication would generate political pressure for system reform that the institutions have not chosen to undertake.

What a disabled Albertan's life will look like after the death of their primary caregiver depends on a set of variables the caregiver, in most cases, cannot fully evaluate during their lifetime. It depends on whether the disabled adult has been deemed eligible for the Persons with Developmental Disabilities (PDD) program — a determination subject to the threshold criteria examined in Section V, which exclude a substantial population of disabled adults whose support needs are real but whose presentations do not match PDD's intellectual-disability framework. It depends on whether residential supports are available within the disabled adult's geographic region — supports whose availability is governed by institutional capacity, contractual relationships between the province and supported-living service providers, and waitlists that are not publicly documented in a form prospective applicants can review. It depends on whether the disabled adult will be placed in a supported-living

arrangement that maintains their connection to community, family, and routine, or in an arrangement that disrupts those connections in the name of administrative efficiency. It depends, in many documented cases, on whether the family of the disabled adult has the resources, time, and institutional knowledge to advocate during the transition — resources that the family of the disabled adult, by definition, no longer has, because the person who held those resources for them has died.

The information available to a primary caregiver attempting to plan for this transition is, by every available measure, inadequate to the planning task. The province does not publish a plain-language guide for parents and partners of disabled adults addressing the question of what will happen after the caregiver's death. The Office of the Public Guardian and Trustee, the institution legally responsible for representing adults who cannot represent themselves and whose families cannot or will not represent them, does not publish operational information about the standards of care it applies, the frequency of its contact with adults under its guardianship, or the criteria by which it makes residential and care decisions on behalf of those adults. The PDD program does not publish information about typical placement timelines following caregiver death, the proportion of placements that maintain or disrupt family contact, the criteria by which one residential setting is selected over another, or the recourse available to a disabled adult or their remaining family members if the placement is unsuitable.

The legal mechanisms by which a caregiver might attempt to secure the disabled adult's future — wills with disability trust provisions, supplementary needs trusts, the Registered Disability Savings Plan (RDSP), guardianship applications, supported decision-making agreements — are documented in legal practice but are not integrated into any provincial public information system that a parent of a disabled child encounters during the years they are raising that child. Parents discover the existence of these mechanisms when they begin actively researching them, typically in middle age when the question of their own mortality becomes concrete, and typically without institutional guidance about which mechanisms apply to their specific circumstances. The cost of consulting a lawyer with disability-trust expertise is not subsidized by the province for families who cannot afford it. The legal

aid framework in Alberta does not include disability succession planning within its standard scope of coverage. The result is a population of caregivers whose ability to secure their disabled family member's future is determined primarily by the family's existing financial resources and existing access to professional networks — that is, by socioeconomic class.

This produces a population of disabled Albertans whose post-caregiver futures are differentiated along class lines that should be irrelevant to the province's care obligations. The disabled adult of an affluent family with access to estate planning, disability trust expertise, and professional advocacy networks enters a structurally different post-caregiver future than the disabled adult of a family without those resources. The province's obligation to the disabled adult, as a citizen, is identical in both cases. The institutional environment the disabled adult will encounter after the caregiver's death is dramatically different. The information vacuum is the mechanism by which this differentiation is produced. A province that published comprehensive succession planning information, integrated it into the FSCD and PDD program contact points families have throughout their disabled child's life, and provided publicly funded legal consultation for disability succession planning would be a province in which class did not determine the post-caregiver future of disabled adults to the degree it currently does. Alberta is not that province. The information is not provided. The class differentiation persists.

The institutional incentive to maintain this information vacuum is documentable. A provincial care system that operates with the residential capacity, staffing levels, and quality standards Alberta currently funds cannot accommodate, at the standards families would consider acceptable, the population of disabled adults whose primary caregivers will die in the next several decades. The demographic curve is clear: a substantial cohort of disabled Albertans whose parents are now in their fifties, sixties, and seventies will require residential and supported-living arrangements within a window the province has not invested in expanding system capacity to meet. The institutional response to this demographic pressure has been, in part, to maintain the information vacuum that prevents families from organizing political pressure for the expansion that would be required. Families who do

not know what their disabled adult's post-caregiver future will look like cannot organize politically around the inadequacy of that future. Families who learn what the future will look like only at the moment of their own incapacitation or death are families who can no longer organize politically at all. The information asymmetry, at this stage of the lifecycle, is operating as a mechanism of political demobilization.

This is the stage at which the institutional-blame analysis the document has been building reaches its full articulation. The lifecycle pattern is not a series of unrelated information failures across stages of disabled life. It is a single coherent operating principle, applied consistently across the lifecycle, by an institutional architecture that benefits from its application. The institution does not communicate the information that would allow the affected population to evaluate the institution's adequacy. The institution does not communicate the information that would allow the affected population to organize politically for reform. The institution does not communicate the information that would allow the affected population to prepare for the foreseeable consequences of the institution's existing inadequacy. The institution does not communicate the information that would allow the affected population to consent meaningfully to the policy transitions the institution implements. At each stage of the lifecycle, the information vacuum operates in a direction that serves the institution's interest in administrative continuity, fiscal predictability, and political stability.

The Sexual Sterilization Act operated under an explicit framework that disabled people were not capable of making decisions about their own lives and that the institutions of the province were therefore entitled to make those decisions for them. The legal architecture of that framework was repealed in 1972. The operational principle it expressed — that disabled Albertans, and the families who support them, are not the primary users of the information governing their own lives — has continued to operate without modification across every disability program and every life stage examined in this report. The mechanism has shifted. The function has not.

The information asymmetry the province maintains is not a bureaucratic accident, an unfunded mandate, or a defensible administrative simplification. It is the operating principle of a system designed to govern a population

whose participation it does not seek, whose consent it does not require, and whose fate after the death of those who advocate for them most fiercely it has no operational obligation to disclose. This is the lifecycle pattern at its full extent, and this is the stage at which it can no longer be characterized as anything else.

There is one further dimension of the lifecycle pattern that this report does not develop in full but cannot leave unnamed. The information asymmetry the province maintains, the income reductions the transition implements, the appeal rights the legislation eliminates, and the labour-market reclassification the Medical Review Panel will conduct do not operate in isolation from the federal legal framework that, since the March 2021 expansion of medical assistance in dying to persons whose natural death is not reasonably foreseeable, has made the deliberate ending of one's life an institutionally available option for disabled Canadians whose conditions are not terminal. Health Canada's annual monitoring reports document a population of Track 2 MAID requests in which financial precarity, housing instability, and inadequate support are cited by the recipient as primary drivers of the request — not the underlying medical condition. Major Canadian disability organizations, including Inclusion Canada and the Council of Canadians with Disabilities, and the United Nations Special Rapporteur on the Rights of Persons with Disabilities, have characterized the intersection of expanded MAID eligibility with reduced disability supports as a circumstance of structural coercion. A government that proceeds with reductions to disability supports in the face of this documented pattern, without conducting a public health impact assessment of the foreseeable interaction between the policy change and disability MAID requests, has accepted the foreseeable outcome as a feature rather than a defect of the policy. This argument is developed in full in the campaign's forthcoming companion document. It is named here because the lifecycle pattern this report has documented terminates not only at succession after the death of a primary caregiver but at the death of the disabled person themselves, in circumstances the province has the operational capacity to ameliorate and has chosen not to.¹³

¹³Health Canada, Annual Reports on Medical Assistance in Dying in Canada (most recent available, [canada.ca](https://www.canada.ca)); Inclusion Canada and Council of Canadians with Disabilities, public position statements on the disability-MAID intersection, 2022-2024; UN Special

The closing argument follows.

Rapporteur on the Rights of Persons with Disabilities, Communications to Canada (Catalina Devandas Aguilar, formerly; Gerard Quinn, formerly; Heba Hagrass, current), 2019–2024. The structural-coercion framing developed in this paragraph is the framing developed by these organizations and is being adopted rather than originated by this report. The intersection of expanded medical assistance in dying eligibility with reduced disability supports is the focus of a forthcoming standalone document in The Alberta Disability System Breakdown campaign series.

VIII.

What Is Owed: Information as the Precondition for Consent

The argument this report has constructed is not, in its conclusion, a policy preference. It is a constitutional and treaty argument. The province of Alberta administers a disability system whose operating principle, demonstrated across every stage of the disabled lifecycle examined in the foregoing sections, is the systematic withholding of information from the population the system governs. This withholding is not incidental to the system's operation. It is the mechanism by which the system maintains administrative continuity, fiscal predictability, and political stability against the pressure for reform that comprehensive disclosure would generate. The withholding has a name in constitutional law and in international human rights law. The name is the absence of informed consent, and the absence is not a defect that can be remedied by additional outreach. It is the precondition the system has chosen to operate under.

The Canadian Charter of Rights and Freedoms

The Canadian Charter of Rights and Freedoms protects, at section 7, the right of every person to life, liberty, and security of the person, and the right not to be deprived thereof except in accordance with the principles of fundamental justice. The principles of fundamental justice, as developed across thirty years of Canadian constitutional jurisprudence including the Supreme Court's analysis in *Baker v. Canada* and the security-of-the-person framework articulated in *Chaoulli v. Quebec*,¹⁴¹⁵

¹⁴*Baker v. Canada* (Minister of Citizenship and Immigration), [1999] 2 S.C.R. 817 (Supreme Court of Canada). Established that the duty of procedural fairness in administrative decision-making varies with the importance of the decision to the affected person, and includes the right to know the case being made, the right to respond, and the right to reasons for decisions affecting fundamental interests.

¹⁵*Chaoulli v. Quebec* (Attorney General), [2005] 1 S.C.R. 791 (Supreme Court of Canada). Held that government action threatening physical and psychological integrity engages Charter section 7 security of the person. Cited in *The Alberta Disability System Breakdown, Charter of Rights Plain Language Guide* (April 2026), as the foundation for the security-of-the-person argument applied to the AISH-to-ADAP income reduction.

include the principle that the state cannot deprive a person of life, liberty, or security of the person on the basis of evidence the person is not permitted to see, by criteria the person is not permitted to review, through processes the person cannot meaningfully participate in. The AISH-to-ADAP transition under Bill 12 deprives approximately 50,000 disabled Albertans of the income security they currently rely on for survival, on the basis of operational rules that have not been published, by criteria that have not been disclosed, through a process the affected population has not been consulted in. Whether this constitutes a section 7 violation is a question that will be litigated, in time, by the disabled Albertans whose section 7 rights the transition implicates. The structural argument this report has built is the evidentiary basis on which that litigation will proceed.

Section 15 of the Charter protects the right of every individual to equal protection and equal benefit of the law without discrimination, and explicitly enumerates mental or physical disability as a protected ground. Equal benefit of the law has been interpreted by the Supreme Court of Canada in *Eldridge v. British Columbia* to require that the substantive content of a law's operation produce equal benefit, not merely that the formal text apply uniformly.¹⁶

A disability system whose information environment produces dramatically different outcomes for disabled Albertans depending on the family resources, professional networks, and personal capacity available to navigate the information vacuum is a system that does not produce equal benefit. The class-differentiated outcomes documented in Section VII of this report are not an unfortunate side effect of administrative complexity. They are the direct product of an information environment the province has chosen not to standardize, and they fall on the protected ground the Charter specifically enumerates.

¹⁶*Eldridge v. British Columbia (Attorney General)*, [1997] 3 S.C.R. 624 (Supreme Court of Canada). Held that the equal benefit guarantee under Charter section 15 requires substantive rather than merely formal equality, and that government failure to provide accommodation in the delivery of public services may itself constitute discrimination on the basis of disability. Foundational for the analysis of disability and the substantive content of equal benefit.

Section 1 of the Charter permits limits on protected rights only where those limits can be demonstrably justified in a free and democratic society. The Oakes test framework requires that the limiting law have a pressing and substantial objective, that the means employed be rationally connected to that objective, that the means impair the protected rights as little as possible, and that the salutary effects outweigh the deleterious effects.¹⁷

The province bears the burden of that demonstration. The province has not demonstrated, and on the existing record cannot demonstrate, that the systematic information vacuum across the disability lifecycle examined in this report serves a pressing and substantial objective, that the means employed are rationally connected to that objective, that the means impair the protected rights as little as possible, or that the salutary effects outweigh the deleterious effects. The information vacuum cannot survive section 1 scrutiny. It has not been required to.

The UN Convention on the Rights of Persons with Disabilities

The United Nations Convention on the Rights of Persons with Disabilities, ratified by Canada on March 11, 2010 and binding on all levels of Canadian government in their treatment of disabled persons, requires at Article 4(3) that “in the development and implementation of legislation and policies to implement the present Convention, and in other decision-making processes concerning issues relating to persons with disabilities, States Parties shall closely consult with and actively involve persons with disabilities, including children with disabilities, through their representative organizations.” The development and implementation of Bill 12 did not meet this standard. No major Alberta disability advocacy organization recognizes the consultation that occurred as meeting the CRPD standard.¹⁸

¹⁷R. v. Oakes, [1986] 1 S.C.R. 103 (Supreme Court of Canada). Established the analytical framework for assessing whether a Charter rights limitation can be “demonstrably justified in a free and democratic society” under section 1. The test requires (a) a pressing and substantial objective; (b) rational connection between means and objective; (c) minimal impairment of the protected right; (d) proportionality between salutary and deleterious effects.

¹⁸Inclusion Alberta, “Bill 12 erodes rights and deepens poverty for Albertans with disabilities,” Media Release, November 26, 2025. See also Disability Action Hall public statements; Alberta Medical Association, Alberta Doctors’ Digest, November/December 2025 (Dr. Brian Wirzba, AMA President); AUPE statements throughout 2025–2026; the Inclusion Alberta documentation that the January 2025 ministry meetings came “too late to be genuine consultations” as discussed in the campaign’s Consultation Gap Rebuttal (April

Inclusion Alberta's media release of November 26, 2025 stated explicitly that "Bill 12 erodes rights and deepens poverty for Albertans with disabilities." The Disability Action Hall, the Alberta Medical Association, and AUPE have publicly opposed the transition. The province ratified the CRPD. The province is bound by Article 4(3). The province has not complied with it.

CRPD Article 12 protects the right of persons with disabilities to recognition everywhere as persons before the law, on an equal basis with others. CRPD Article 13 protects the right to access to justice for persons with disabilities. Article 19 protects the right of disabled persons to live independently and to be included in the community, with the full range of in-home, residential, and other community support services. Article 21 protects the right of access to information, including the obligation of States Parties to provide information intended for the general public to persons with disabilities in accessible formats and technologies appropriate to different kinds of disability, in a timely manner and without additional cost. Article 28 protects the right to an adequate standard of living and social protection, including the right to the continuous improvement of living conditions. Each of these rights is implicated by the operational practices examined in this report. The reversal of 79,290 prior findings of permanent disability without individual reassessment implicates Article 12. The elimination of independent appeal under Bill 12 implicates Article 13. The opacity of post-caregiver succession planning implicates Article 19. The absence of plain-language documentation of program operational rules implicates Article 21. The transition from AISH benefit levels to ADAP benefit levels at sub-survival thresholds implicates Article 28. The province has ratified all of these. The province is bound by all of these. The province has not been held to any of these.

The UN Committee on the Rights of Persons with Disabilities issued its Concluding Observations on Canada on March 26, 2025 — nine months before Bill 12 passed. The Committee specifically recommended that Canada "establish legally defined mechanisms ensuring compliance with the CRPD by all provinces and territories, and condition fiscal transfers from the Federal level to provinces and territories on their adherence to the CRPD."¹⁹

2026).

¹⁹UN Committee on the Rights of Persons with Disabilities, Concluding Observations on the combined second and third periodic reports of Canada, March 26, 2025,

Canada has not acted on this recommendation. Alberta's Bill 12 is the direct and documented consequence of that inaction. The province has been told by the United Nations, through the federal government that ratified the CRPD on the province's behalf, that fiscal transfers from the federal level should be conditioned on provincial adherence to the Convention. The province has nonetheless legislated in direct contradiction of the Convention, in the period following the publication of the Committee's recommendation, with no consequence to the federal funding the province continues to receive.

The Alberta Bill of Rights

The Alberta Bill of Rights, the province's own foundational rights instrument, protects at section 1(a) the right of the individual to "liberty, security of the person and enjoyment of property, and the right not to be deprived thereof except by due process of law."²⁰

Due process of law has, across centuries of Anglo-Canadian jurisprudence, included the right to know the case being made against one, to review the evidence on which decisions affecting one's interests are based, and to participate in the proceedings through which those decisions are made. The AISH-to-ADAP transition meets none of these standards for the affected population. The province's own rights instrument is being violated by the province's own administration of the program the instrument was designed to constrain.

What Is Owed

What disabled Albertans are owed, therefore, is not a policy adjustment. It is the constitutional and treaty obligations the province has already accepted. Specifically:

CRPD/C/CAN/CO/2-3. The Committee's review involved 53 disability organizations, 14 of which travelled to Geneva to present in person. The recommendation cited is the Committee's formal recommendation to Canada that fiscal transfers from the federal level to provinces and territories be conditioned on adherence to the CRPD. Bill 12 was passed by the Alberta Legislature on December 9, 2025 — nine months after this recommendation was issued.

²⁰Alberta Bill of Rights, R.S.A. 2000, c. A-14, section 1(a). The province's own foundational rights instrument, predating the Canadian Charter of Rights and Freedoms by over twenty years.

The full operational rules of the Alberta Disability Assistance Program, in plain-language form, published in advance of any individual recipient's transition to ADAP, with a meaningful comment period in which affected individuals and their representative organizations can review and respond to the rules before they take effect.

The full eligibility criteria, assessment methodology, and adjudication standards used by the medical assessors who will determine ADAP placement, published in a form accessible to applicants in advance of their assessment, with the right of the applicant to know which criteria were applied to their assessment and the reasoning by which the assessor reached their determination.

The continuation period for supplementary health benefits during and after the AISH-to-ADAP transition, published with specificity, including the formulary of medications, devices, and services covered under the new structure, before any recipient is required to consent to the transition.

The restoration of independent appeal rights for AISH eligibility decisions through a panel structurally and operationally independent of the ministry administering the program — not a return specifically to the Citizens Appeal Panel, but the maintenance of the principle of independent appeal that the Citizens Appeal Panel embodied and that Bill 12 has eliminated.

The integration of comprehensive disability information into the public health and educational systems of the province, including prenatal and reproductive education, early childhood developmental milestone documentation, and adult diagnostic pathway disclosure, such that families and individuals encountering disability at any stage of the lifecycle do so with the institutional support of the information the province has been collecting and not communicating.

The publication, in plain-language form, of post-caregiver succession planning information for parents and partners of disabled adults, including residential placement processes, guardianship structures,

supported decision-making frameworks, and the legal mechanisms by which a caregiver can secure their disabled family member's future, integrated into the FSCD and PDD program contact points families have throughout their disabled child's life.

The acknowledgment, in any future policy reform process affecting disabled Albertans, that the province's consultation obligations under CRPD Article 4(3) require not the appearance of consultation through engagement sessions whose outcomes the province has predetermined, but the substantive participation of disabled Albertans and their representative organizations in the design of the policies that will govern their lives.

These are not policy preferences. They are the conditions the province has, in its constitutional order and in its international treaty commitments, already obligated itself to meet. The transition currently being implemented under Bill 12 violates these conditions. The disability administration system across the lifecycle this report has examined violates these conditions. The province has chosen, across decades, not to be held to the obligations it has accepted. The disabled Albertans this report addresses are owed the obligations that have been accepted on their behalf and not delivered to them.

The repeal of the Sexual Sterilization Act in 1972 ended one expression of the institutional principle that disabled Albertans are not entitled to participate in decisions affecting their own lives. The principle did not end with the repeal. It continued to operate, as this report has documented, through the lifecycle of every disabled Albertan since. The transition currently being implemented under Bill 12 is the most recent expression. It will not be the last unless the principle itself is named, identified, and structurally addressed.

This report is the naming.

About This Report

This report is part of a series produced by The Alberta Disability System Breakdown campaign, alongside The Government's Case Against Itself (April 2026) and the planned Alberta Disability Eugenics historical analysis (forthcoming, summer 2026).

All factual claims in this report are drawn from publicly available primary sources, documented in the footnotes and collected in the Sources and References index that follows. Primary sources include: Government of Alberta program documentation; the AISH Policy Manual; the AISH Act; Bill 12 (Financial Statutes Amendment Act, 2025); the Canadian Charter of Rights and Freedoms; the United Nations Convention on the Rights of Persons with Disabilities; the Concluding Observations of the UN Committee on the Rights of Persons with Disabilities on Canada (March 26, 2025); the Alberta Bill of Rights; published media releases from disability advocacy organizations including Inclusion Alberta and the Disability Action Hall; peer-reviewed medical and disability studies literature; and the documented operational practices of provincial ministries as reported by recipients, advocates, and frontline workers.

This report makes no claim to neutrality. It is written by an AISH recipient who has been continuously eligible since April 1, 2016, and who is one of the disabled Albertans being transitioned to ADAP on July 1, 2026. The structural argument advanced in this report is informed by lived experience, by the lived experiences of the community members of The Alberta Disability System Breakdown group, and by the public record of the institutions whose practices the report describes.

The report does not represent the official position of any organization other than The Alberta Disability System Breakdown campaign, of which the author is the founder and administrator. The report is offered for free public distribution. It may be cited, quoted, shared, and reproduced without permission, with attribution.

Author and Campaign

The Alberta Disability System Breakdown — A public information and advocacy campaign concerning the Alberta disability system and the AISH-to-ADAP transition, founded April 6, 2026.

Authorship. This report is published by the campaign as a collective work. It draws on the lived experience of an AISH recipient continuously eligible since April 1, 2016, on the public record of provincial and federal disability administration, and on the documented experiences of community members. The campaign is the responsible party.

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Sources and References

All citations in this report are drawn from publicly available primary sources. Footnotes appear with each substantive claim. The index below collects the full source apparatus, organized by category for ease of reference. Cross-references to companion documents in The Alberta Disability System Breakdown campaign archive are noted where directly relevant.

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